Oppositional Defiant Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

Brian Campbell, Ph.D.

Prepared with AI research and drafting assistance; content directed, reviewed, and approved by the author. © 2026 Brian Campbell, Ph.D. All rights reserved.

1. What it is

Oppositional defiant disorder (ODD) is a persistent pattern of angry or irritable mood, argumentative or defiant behavior, and vindictiveness that goes well beyond ordinary childhood stubbornness and causes real problems at home, at school, or with peers (American Psychiatric Association [APA], 2022). It sits in DSM-5-TR's "Disruptive, Impulse-Control, and Conduct Disorders" chapter, alongside conduct disorder.

How common: across 41 community studies in 27 countries, about 5.7% of children and adolescents had a disruptive behavior disorder (ODD or conduct disorder) (Polanczyk et al., 2015).

Two sides of ODD. Following 1,420 young people from ages 9 to 21, researchers found that ODD symptoms split into an **irritable** dimension and a **headstrong** dimension. ODD more strongly predicted later **emotional** disorders (anxiety and depression) in young adulthood, while conduct disorder mainly predicted later behavior problems. ODD led on to conduct disorder in boys, but not in girls (Rowe et al., 2010).

Counselor framing: ODD is not simply a "strong-willed child." It is a relationship pattern in which a child's irritability and the adults' responses feed each other. The most effective help works through the parents.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pattern lasting **at least 6 months**, with **4 or more** of the following, shown with at least one person who is not a sibling:

- **Angry or irritable mood:** often loses temper; often touchy or easily annoyed; often angry and resentful.
- **Argumentative or defiant behavior:** often argues with adults or authority figures; often actively defies or refuses rules or requests; often deliberately annoys others; often blames others for his or her own mistakes or misbehavior.
- **Vindictiveness:** spiteful or vindictive at least twice in the past 6 months.
- **Frequency guide:** for children under 5, most days for 6 months; for age 5 and older, at least once a week for 6 months. Severity is rated mild (one setting), moderate (two settings), or severe (three or more settings).

Rule out first: normal developmental defiance, ADHD (which often co-occurs), depression or bipolar disorder (irritability can be a mood symptom), disruptive mood dysregulation disorder, language or learning problems, and reactions to abuse, trauma, or chaotic homes. If the child also violates others' basic rights (aggression to people or animals, destruction, serious theft, running away), consider conduct disorder (APA, 2022).

When counselors must refer (general clinical consensus): refer for medical and psychiatric evaluation when ADHD, a mood disorder, developmental delay, or substance use is suspected; when there is self-harm or suicidal talk; or when you suspect abuse or neglect, which also triggers mandated reporting.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in oppositional defiant disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
APA Clinician-Rated Severity of Oppositional Defiant Disorder	Rating ODD severity across settings.	Clinician-rated, brief	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
ARI (Affective Reactivity Index)	Irritability in children and teens ages 6 to 17; an APA DSM-5 Level 2 measure.	Parent- and self-report, 7 items	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
NICHQ Vanderbilt Assessment Scales	Parent and teacher ratings of ADHD, ODD, conduct, anxiety, and depression symptoms, ages 6 to 12.	Parent and teacher forms, initial and follow-up	No (free)	None beyond general clinical training	NICHQ (PDF)
SDQ (Strengths and Difficulties Questionnaire)	Brief screen of conduct, emotional, attention, and peer problems plus strengths.	Parent, teacher, and self-report, 25 items	No (free paper version)	None beyond general clinical training	SDQ website
ECBI / SESBI-R (Eyberg Child Behavior Inventory)	Frequency of disruptive behavior at home and school, ages 2 to 16; tracks progress in parent training.	Parent form 36 items; teacher form 38 items	Yes	★ Level B	PAR
BASC-3 (Behavior Assessment System for Children)	Broad behavioral and emotional problems and strengths, ages 2 to 21 (college self-report to 25).	Parent, teacher, and self-report forms	Yes	★ Level B	Pearson
Conners 4	ADHD and related problems, ages 6 to 18.	Parent, teacher, and self-report forms (full, short, 12-item index)	Yes	★ Level B	MHS

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: behavioral parent training is the first-line treatment for ODD. Cognitive-behavioral skills work with the child adds benefit, especially for older children. Medication does not treat ODD itself.

Evidence overview (honest effect sizes):

- **Parent training is best established:** for children 12 and under, group parent behavior therapy and individual parent behavior therapy with the child taking part were the two treatments rated "well-established"; 13 others were "probably efficacious" (Kaminski & Claussen, 2017).
- **Specific programs:** both Parent-Child Interaction Therapy (PCIT) and Triple P–Positive Parenting Program improved child behavior, with effects varying by program length, components, and who rated the outcome (Thomas & Zimmer-Gembeck, 2007). Across 50 studies of the Incredible Years parent program, the overall effect on disruptive behavior was small (d = 0.27) but rose to moderate (d = 0.50) in treatment studies of children who already had significant problems; the more severe the starting behavior, the larger the benefit (Menting et al., 2013).
- **CBT works in ordinary clinics:** across 51 studies (5,295 patients) of CBT-based treatment for externalizing disorders delivered in routine care, children with ODD or conduct disorder improved substantially (within-group g = 0.98), 48% remitted, and only 14% dropped out, results similar to university trials (Riise et al., 2021).
- **Medication (referral notes):** when ADHD is also present, stimulant medication had a moderate-to-large effect on oppositional behavior, conduct problems, and aggression (high-quality evidence); atomoxetine and guanfacine had smaller effects (Pringsheim et al., 2015). There is no medication for ODD alone.

Parent training and CBT for ODD: how they are used (Kaminski & Claussen, 2017; Riise et al., 2021):

- **Change the cycle.** Increase warm, child-led time and specific praise; give clear, calm commands; ignore minor attention-seeking; use brief, predictable consequences such as time-out instead of yelling or hitting.
- **Identify and challenge parent beliefs** that fuel harsh cycles: "He does it on purpose to make me angry," "If I give an inch she'll take a mile," "Only spanking works," "I am a failure as a parent."
- **Teach the child skills:** naming anger, calming down, solving problems, and seeing others' intentions more accurately ("Everyone is against me" → "Mom said no to the snack, not to me").

© 2026 Brian Campbell, Ph.D. All rights reserved.

5. Religion, spirituality, and Christian integration: what the evidence shows

Harsh discipline backfires, including in Christian homes. Across 111 effect sizes representing 160,927 children, spanking was significantly linked to **harmful** child outcomes in 13 of 17 analyses, and none of those findings favored spanking (Gershoff & Grogan-Kaylor, 2016). Counselors serving Christian families will often need to address this directly and respectfully.

Seeing parenting as sacred can help. In 58 families with preschoolers, mothers and fathers who viewed parenting as a God-given, sacred task used more praise, more positive socialization, and more reasoning with the child, and children whose parents both used reasoning showed stronger conscience development (Volling et al., 2009).

Church-delivered parent training. In a pilot study across six Korean American churches (48 mothers), a 12-week parent-training program reduced harsh discipline and mother-reported child behavior problems compared with waiting-list churches (Kim et al., 2014). In a pilot randomized trial of an evidence-based parenting program held in Filipino American churches (28 parents), physical punishment and parenting stress decreased, and child behavior problems improved (Javier et al., 2016). These programs were **delivered in** churches; their content was standard, not Christian-specific.

Broader evidence. Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018); none of those studies focused on ODD.

Honest summary for teaching:

1. Behavioral parent training is the evidence base. Faith integration supports it; it does not replace it.
2. Churches are a promising, trusted setting for delivering proven parenting programs, based on small pilot studies.
3. No trial of Christian-integrated treatment for ODD exists. Physical punishment is linked to worse behavior; gentle, consistent discipline is both biblical and evidence-based.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within parent training. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for parent beliefs: "A defiant child is a rebellious sinner who must be broken," "God will judge me for my child's behavior," "Spare the rod, spoil the child means I must spank." Listen for the child's beliefs: "Nobody listens to me," "I'm the bad kid," "God is angry with me too" (Gershoff & Grogan-Kaylor, 2016).
- **Challenge (clinical).** Test what actually happens at home when praise and calm consequences replace yelling and spanking. Use a simple chart so parents can see the evidence for themselves (Kaminski & Claussen, 2017). **Possible cognitive errors** to watch for in children and in parents, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
    - **Blaming others** — others are responsible for how I feel. *Feeds:* irritability and blaming. *Sounds like:* "He made me mad."
    - **Overgeneralization** — one event becomes a rule. *Feeds:* resentment. *Sounds like:* "Nobody ever listens to me."
    - **Jumping to conclusions** — deciding without the facts. *Feeds:* anger and spitefulness. *Sounds like:* "She did that to annoy me."
    - **Global labeling** — one mistake becomes who I am. *Feeds:* defiance and low self-worth. *Sounds like:* "I'm the bad kid."
    - **All-or-nothing thinking** — no middle ground. *Feeds:* harsh parenting (in parents). *Sounds like:* "Either I break his will or he wins."
    - **Personalization** — taking blame for things outside my control. *Feeds:* parental guilt. *Sounds like:* "God will judge me for my child's behavior."
    - **Fortune telling** — predicting a bad outcome. *Feeds:* parents giving up on new methods. *Sounds like:* "Praise won't work with this kid."
- **Questions to dispute the belief.** Have the parent (or an older child) choose questions from the Questioning False Beliefs form that "speak to" the belief. To see how, watch Dr. Campbell's video Integrated Christian Counseling: Questioning Your Beliefs. Selected questions that fit the errors above:
    - "Are there other ways of looking at this situation?" *For:* blaming others and jumping to conclusions.
    - "Is it possible that I am distorting the facts?" *For:* overgeneralization.

© 2026 Brian Campbell, Ph.D. All rights reserved.

- "Doesn't everyone have good points and bad points?" and "How might an all-loving God look at me or this situation?" *For:* global labeling and "God is angry with me too".
- "Am I keeping things in perspective?" *For:* all-or-nothing thinking.
- "Am I being too hard on myself?" *For:* a parent's personalization.
- "What's the actual evidence for my belief?" *For:* fortune telling (test it with a chart).

- **Replace (pastoral).** Offer Scripture that frames discipline as patient training, not punishment:
  - "Fathers, do not provoke your children to wrath; instead, bring them up in the discipline and instruction of the Lord." (Ephesians 6:4). Discipline that teaches, not discipline that inflames.
  - "Fathers, do not embitter your children, so they will not become discouraged." (Colossians 3:21)
  - "A gentle answer turns away wrath, but a harsh word stirs up anger." (Proverbs 15:1). The parent's calm response breaks the cycle.
  - "No discipline seems enjoyable at the time, but painful. Later on, however, it yields a peaceful harvest of righteousness to those who have been trained by it." (Hebrews 12:11). Consistency over time.
- **Faith community (clinical).** Children's ministry workers can use the same calm, consistent plan the parents use. Churches can host proven parenting programs (Kim et al., 2014; Javier et al., 2016).
- **Ministers:** support exhausted parents without shaming them or the child, and refer when there is aggression, self-harm, or any sign of abuse.

7. Quick classroom takeaways

- ODD is 6 months or more of angry or irritable mood, defiance, and vindictiveness (4 of 8 symptoms). Rule out ADHD, mood disorders, and trauma; ODD can lead to later emotional problems.
- Parent training is first-line; CBT skills help; stimulants help when ADHD is present. Treat the relationship, not just the child.
- Replace harsh, angry discipline with calm, consistent training: "Do not provoke your children to wrath."
- Use the free Vanderbilt or SDQ to screen, and the ECBI to track progress in parent training; rate behavior in more than one setting.

Sources note

All 13 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Gershoff, E. T., & Grogan-Kaylor, A. (2016). Spanking and child outcomes: Old controversies and new meta-analyses. *Journal of Family Psychology*, *30*(4), 453–469. https://doi.org/10.1037/fam0000191

Javier, J. R., Coffey, D. M., Schrager, S. M., Palinkas, L. A., & Miranda, J. (2016). Parenting intervention for prevention of behavioral problems in elementary school-age Filipino-American children: A pilot study in churches. *Journal of Developmental and Behavioral Pediatrics*, *37*(9), 737–745. https://doi.org/10.1097/DBP.0000000000000342

Kaminski, J. W., & Claussen, A. H. (2017). Evidence base update for psychosocial treatments for disruptive behaviors in children. *Journal of Clinical Child and Adolescent Psychology*, *46*(4), 477–499. https://doi.org/10.1080/15374416.2017.1310044

Kim, E., Cain, K., Boutain, D., Chun, J.-J., Kim, S., & Im, H. (2014). Pilot study of the Korean Parent Training Program using a partial group randomized experimental study. *Journal of Child and Adolescent Psychiatric Nursing*, *27*(3), 121–131. https://doi.org/10.1111/jcap.12071

Menting, A. T. A., Orobio de Castro, B., & Matthys, W. (2013). Effectiveness of the Incredible Years parent training to modify disruptive and prosocial child behavior: A meta-analytic review. *Clinical Psychology Review*, *33*(8), 901–913. https://doi.org/10.1016/j.cpr.2013.07.006

Polanczyk, G. V., Salum, G. A., Sugaya, L. S., Caye, A., & Rohde, L. A. (2015). Annual research review: A meta-analysis of the worldwide prevalence of mental disorders in children and adolescents. *Journal of Child Psychology and Psychiatry*, *56*(3), 345–365. https://doi.org/10.1111/jcpp.12381

Pringsheim, T., Hirsch, L., Gardner, D., & Gorman, D. A. (2015). The pharmacological management of oppositional behaviour, conduct problems, and aggression in children and adolescents with attention-deficit hyperactivity disorder, oppositional defiant disorder, and conduct

© 2026 Brian Campbell, Ph.D. All rights reserved.

disorder: A systematic review and meta-analysis. Part 1: Psychostimulants, alpha-2 agonists, and atomoxetine. *The Canadian Journal of Psychiatry*, *60*(2), 42–51. https://doi.org/10.1177/070674371506000202

Riise, E. N., Wergeland, G. J., Njardvik, U., & Öst, L.-G. (2021). Cognitive behavior therapy for externalizing disorders in children and adolescents in routine clinical care: A systematic review and meta-analysis. *Clinical Psychology Review*, *83*, 101954. https://doi.org/10.1016/j.cpr.2020.101954

Rowe, R., Costello, E. J., Angold, A., Copeland, W. E., & Maughan, B. (2010). Developmental pathways in oppositional defiant disorder and conduct disorder. *Journal of Abnormal Psychology*, *119*(4), 726–738. https://doi.org/10.1037/a0020798

Thomas, R., & Zimmer-Gembeck, M. J. (2007). Behavioral outcomes of Parent-Child Interaction Therapy and Triple P—Positive Parenting Program: A review and meta-analysis. *Journal of Abnormal Child Psychology*, *35*(3), 475–495. https://doi.org/10.1007/s10802-007-9104-9

Volling, B. L., Mahoney, A., & Rauer, A. J. (2009). Sanctification of parenting, moral socialization, and young children's conscience development. *Psychology of Religion and Spirituality*, *1*(1), 53–68. https://doi.org/10.1037/a0014958

Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 13 references and DOIs verified via Crossref, OpenAlex, and PubMed/Europe PMC records (Sunday, Nov 1, 2026). Effect sizes as reported in the cited sources.

© 2026 Brian Campbell, Ph.D. All rights reserved.